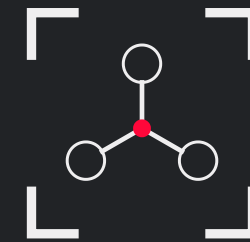


Nosotrack infers who
infected whom in near
real-time, enabling control
teams to stop outbreaks
before they escalate.



NOSOTRACK™

Track.
Intervene.
Protect.

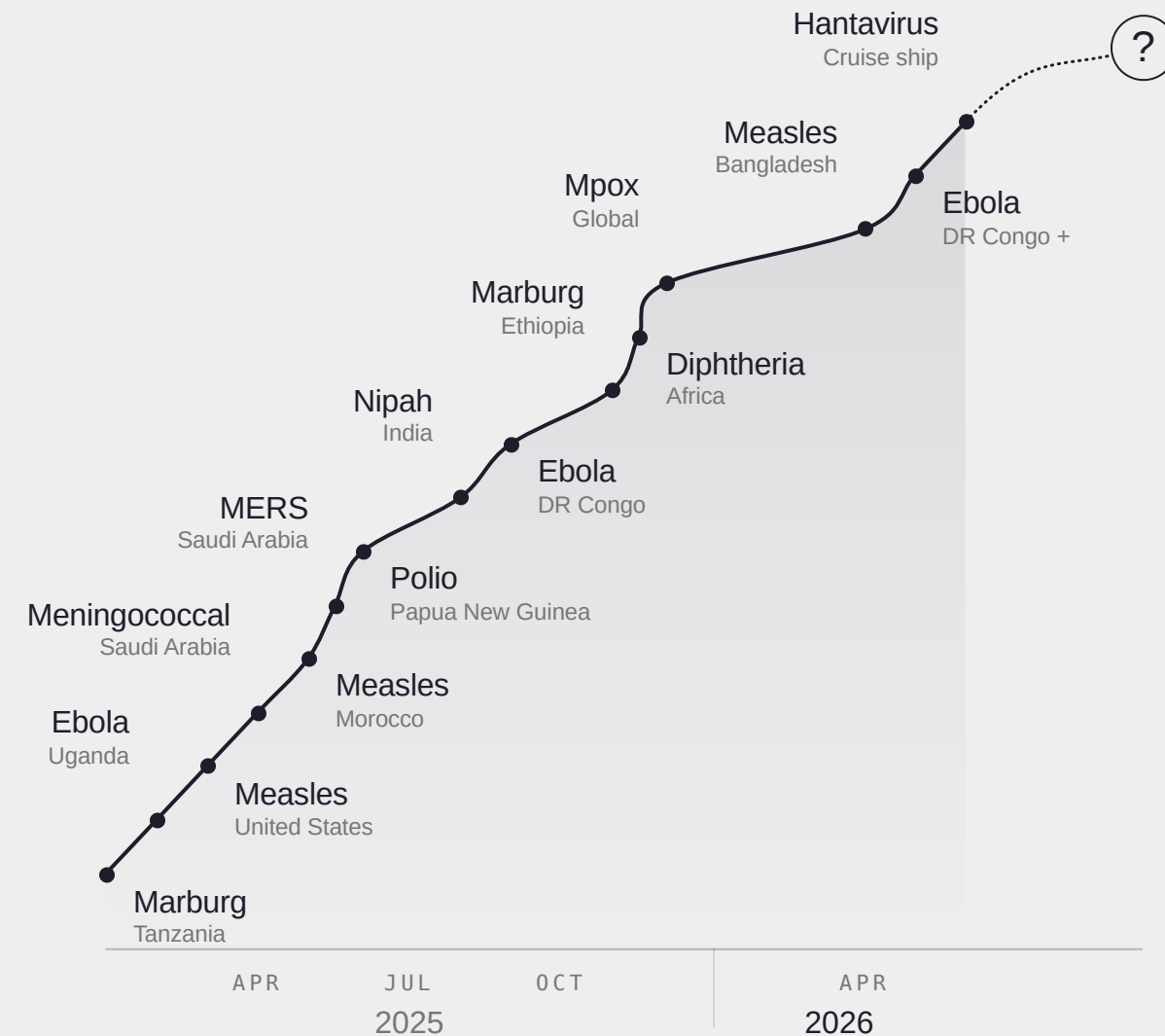
Cyril Geismar

Postdoctoral Research Fellow

Johns Hopkins Bloomberg School of Public Health

Infectious diseases are on the rise.

15 major WHO Disease Outbreak News alerts in the last 18 months.

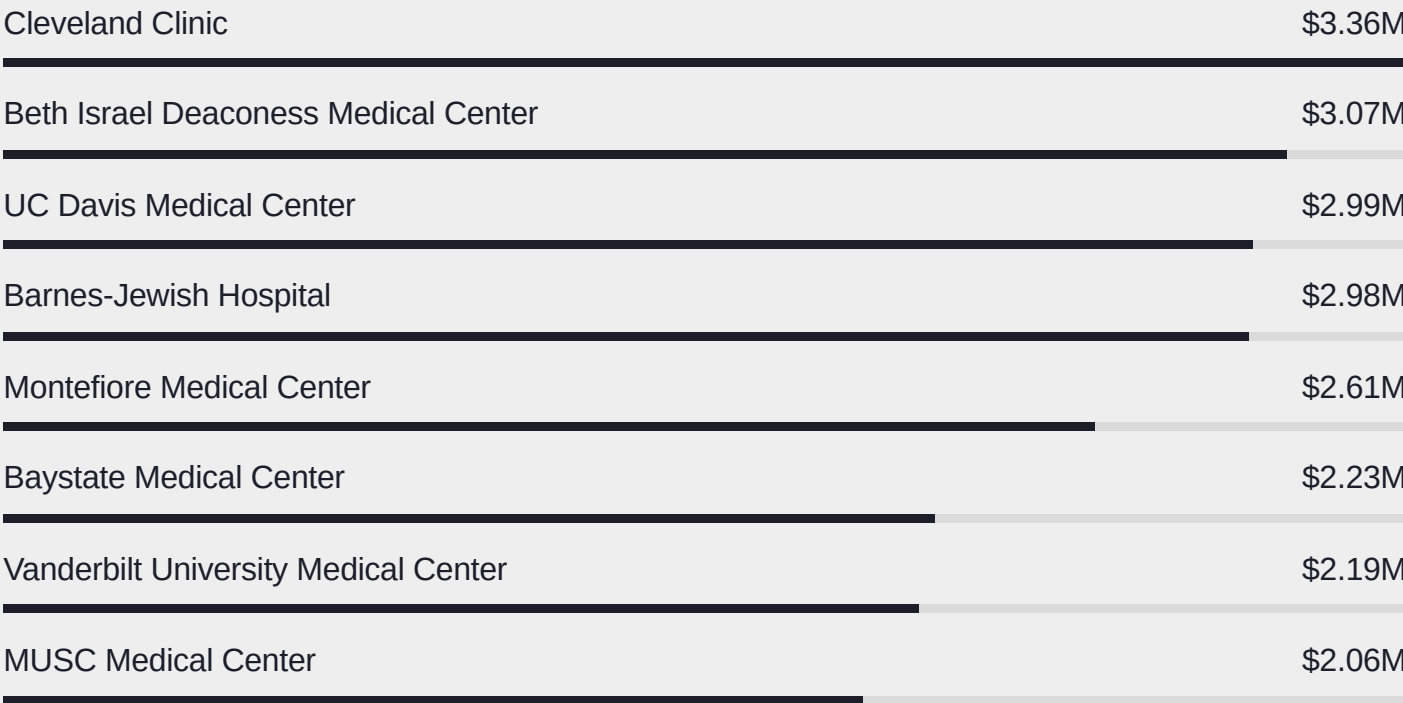


Healthcare-associated infections create major clinical, regulatory and financial risk.

Healthcare-associated infections (HAIs) affect 1 in 10 hospitalised patients worldwide¹ and are a major driver of antimicrobial resistance.

US impact:

- 518K HAIs annually²
- 1 in 38 hospitalised patients affected on any given day²
- HAIs must be reported to CDC’s National Healthcare Safety Network³
- 719 hospitals penalised by Medicare⁴



2026 Medicare HAC penalties · estimated at 1% of Medicare payments⁴

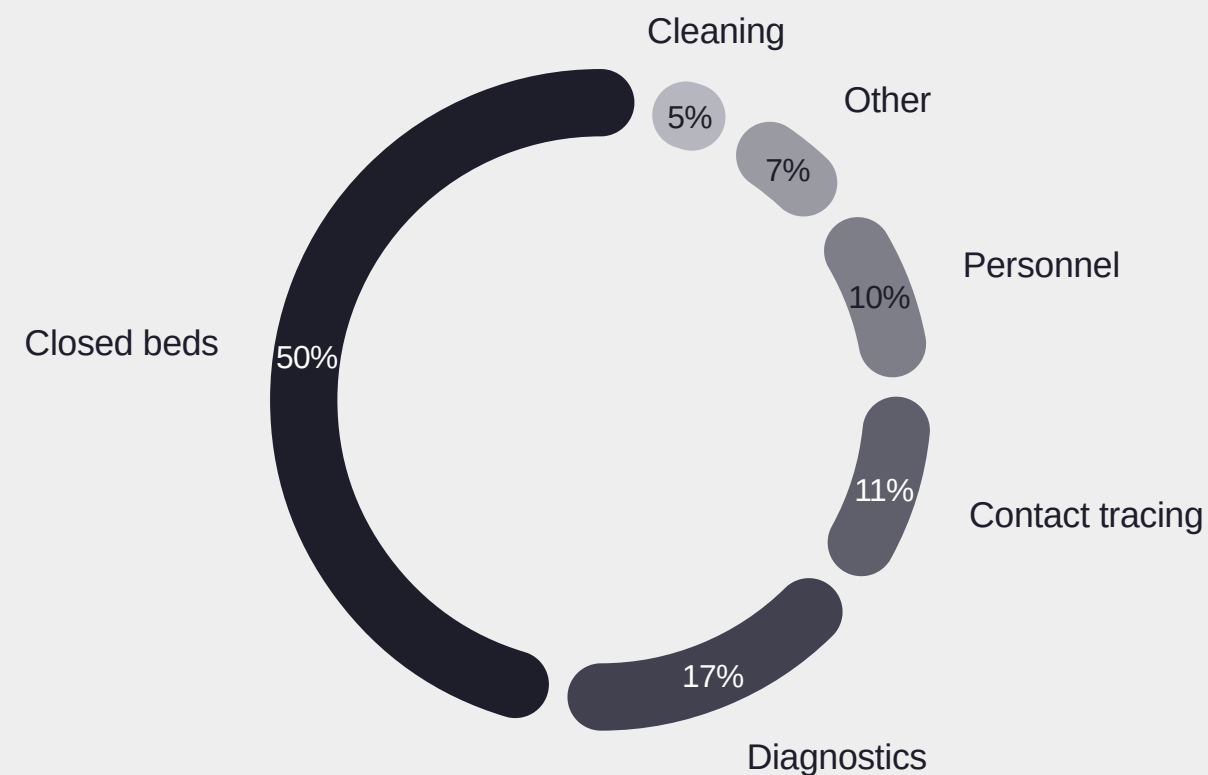
Nosocomial outbreaks are costly and difficult to manage.

Containment costs escalate with outbreak size.

Ward closures, contact tracing and diagnostics can cost hundreds of thousands per outbreak.^{1 2} A single CPE outbreak affecting five London hospitals cost €1.1 million (€0.9–1.4 million) over 10 months.³

Typical hospital outbreak¹

- 12 patients infected (3–37)
- 45 days to contain (16–86)
- €172K containment cost (€11K–€357K)
- \$50K treatment per case (\$31K–\$74K)⁴
- \$105K/year (\$24K–\$214K) for clinical surveillance software without transmission reconstruction⁵



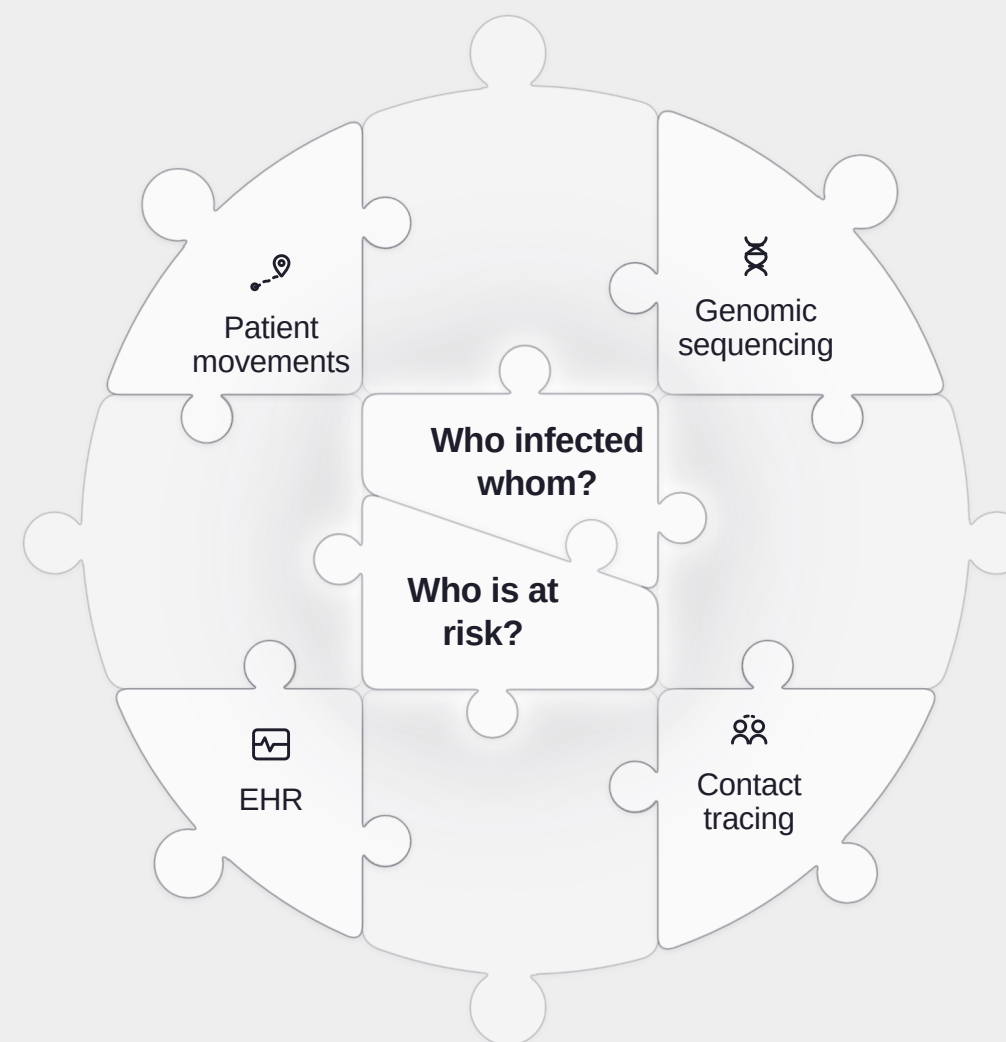
Share of containment expenses¹

Outbreak data are fragmented. The response cannot be.

Outbreak investigation remains largely manual.

Teams piece together contact tracing, clinical records and microbiology results through ad hoc analyses. The process is slow, labour-intensive, inconsistent and difficult to audit.

Nosotrack fuses these data into a unified Bayesian inference engine powered by the open-source, peer-reviewed outbreaker2 model¹² to identify transmission chains in near real time, revealing the drivers of disease transmission and enabling targeted control strategies.

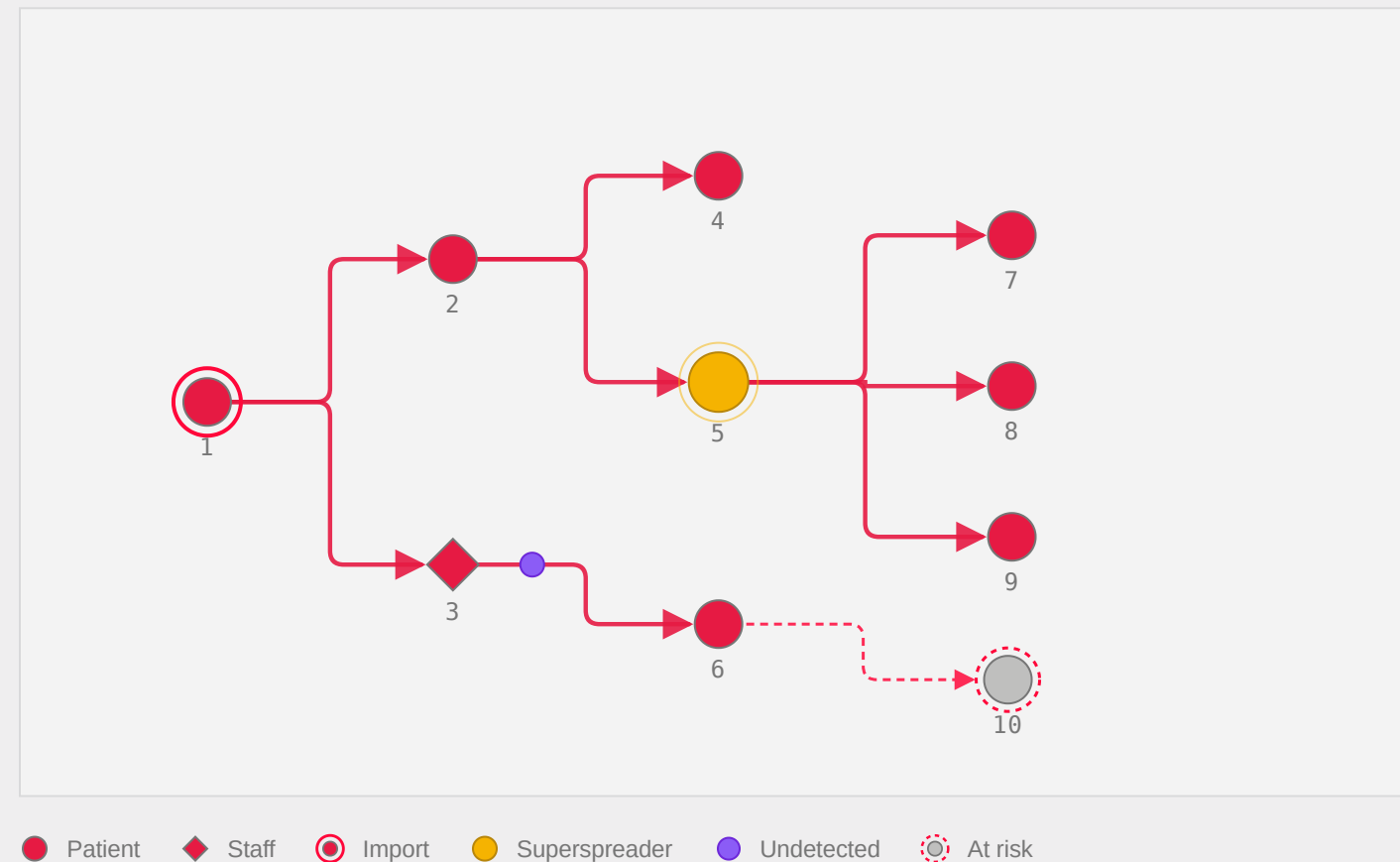


Every outbreak leaves a trail. Nosotrack finds it.

Nosotrack reconstructs the transmission tree.

Identifying the sources of infection and how disease spreads is critical to devising targeted control strategies.

Nosotrack provides a consistent visual interface to the outbreaker2 engine identifying the most likely index case, imported cases, superspreaders, undetected cases, transmission patterns and individuals at risk. Additional developments will help identify potential sources of contamination, such as medical equipment.

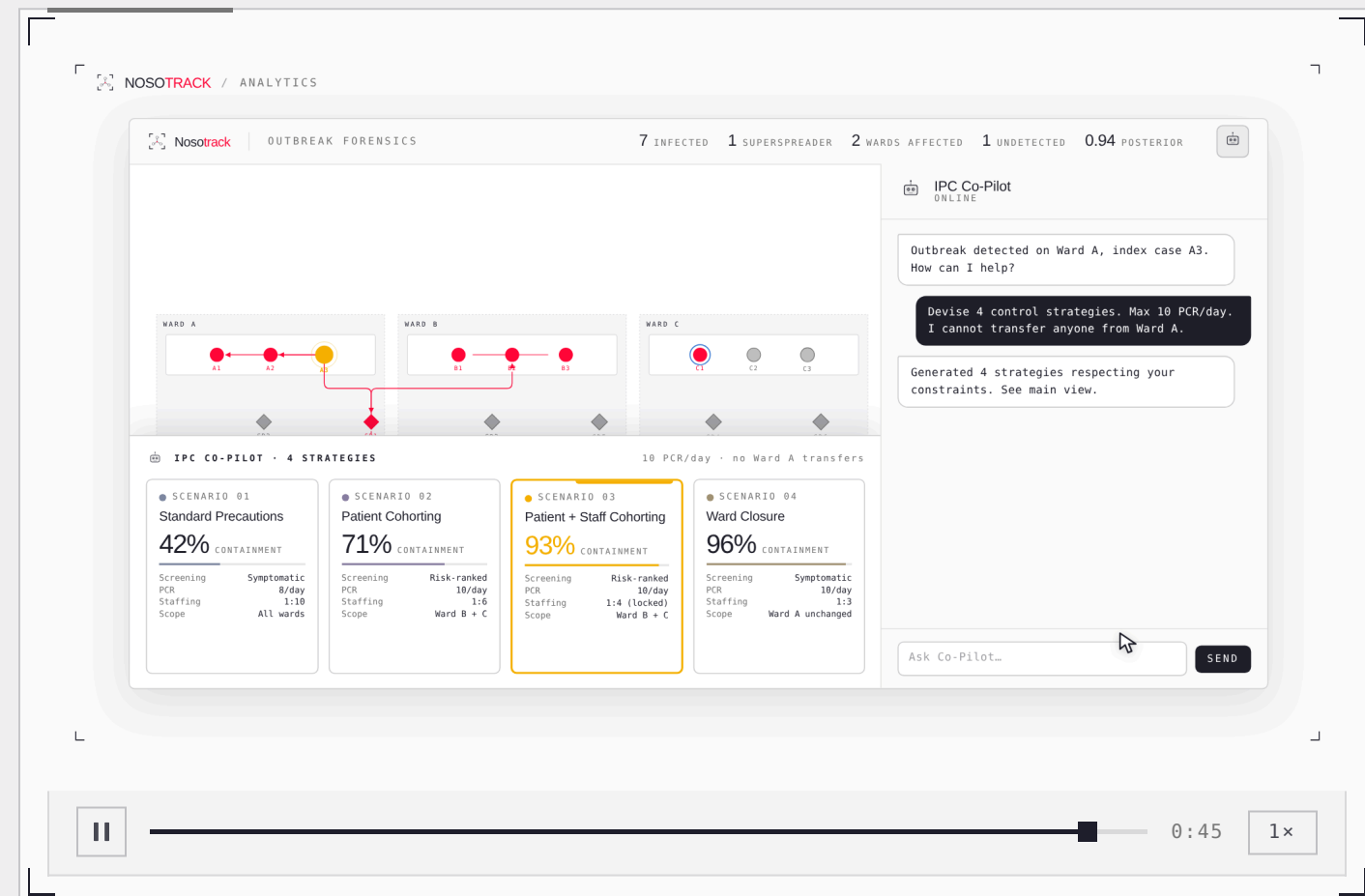


Outbreak forensics, end-to-end.

Integrate: EHR, sequencing, and contact-tracing streams unified into a single engine.

Reconstruct: Bayesian inference reconstructs who infected whom, and who is at risk next.

Intervene: An AI co-pilot scores intervention scenarios in seconds, ranked by containment, cost, and bed-days saved.



Peer-reviewed science, adopted globally.



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LONDON



312,275+
DOWNLOADS

46,785
CITATIONS

127
COUNTRIES

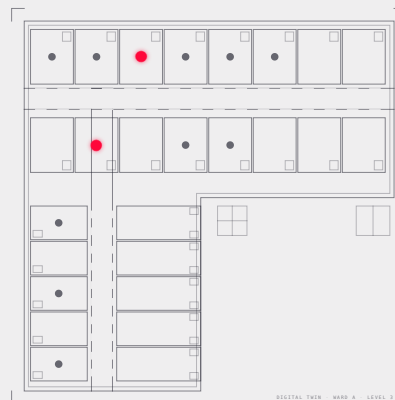
311
PUBLICATIONS

Pathogen agnostic, deployable anywhere.

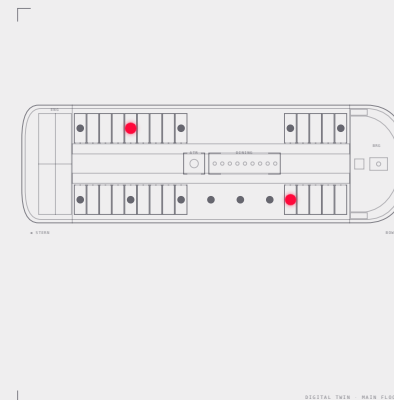
Nosotrack provides the operating system for infectious disease response.

We aim to deliver a complete outbreak intelligence service, combining analytics, contact-tracing hardware, rapid diagnostics, sequencing and field epidemiology. Our technology strives to support outbreak responses across all scales, from Candida auris in hospitals and swine flu on farms to WHO-coordinated responses to global health emergencies such as Ebola.

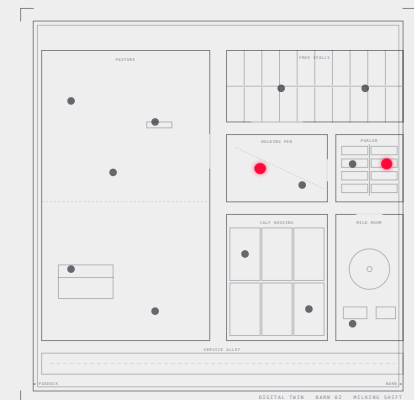
ACUTE CARE HOSPITAL



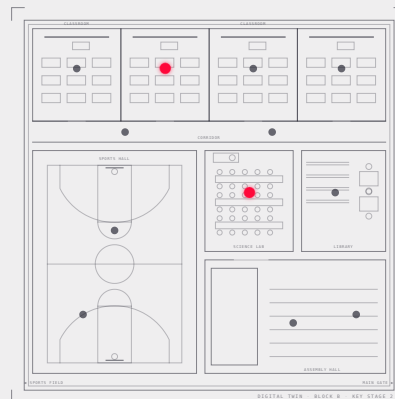
CARIBBEAN CRUISE



DAIRY FARM



PRIMARY SCHOOL



CARE HOME



BUNIA CITY

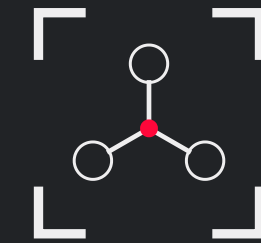


From validated science to first pilot deployment.

Define the MVP with IPC teams at target facilities.

Complete the simulation study validating the core engine across pathogens, settings, and data availability.

Secure the first pilot agreement with a partner hospital.



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